IN THE SUPREME COURT
STATE OF GEORGIA

CHARLES CLARK, Individually, and APRIL D. CLARK, as Administrator of the Estate of April S. Clark, Deceased,
Appellants,

v.

THOMAS LEIGH, M.D., WILLIAM SHIRLEY, M.D., and OB/GYN SPECIALISTS, LLP,
Appellees.

Case No. S26A0349

RESPONSE BRIEF OF APPELLEES

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INTRODUCTION

This appeal asks whether the General Assembly can limit by statute a remedy which it created by statute. Precedent and common sense dictate that it can. Accordingly, the trial court properly held that Georgia's statutory cap on noneconomic damages, O.C.G.A. § 51-13-1, applied to Plaintiff Charles Clark's recovery of noneconomic damages awarded for the statutory remedy of wrongful death, requiring it to reduce its original judgment. Plaintiffs show no error in that decision.

In 2020, Plaintiffs sued Drs. Thomas Leigh and William Shirley (among others) for medical malpractice and wrongful death, arising out of April S. Clark's death in 2019. A jury found in their favor, awarding Charles Clark more than $29 million for his wrongful death claim. In post-judgment motions, Defendants asked the trial court to remit the judgment on that verdict to $350,000, as required by O.C.G.A. § 51-13-1. They acknowledged that this Court had held in *Nestlehutt* that the cap was invalid when applied to some common-law damages. But Defendants argued that *Nestlehutt* did not invalidate the cap as applied to wrongful death. While the motions were pending, this Court agreed with that premise in *Turner*, holding that because *Nestlehutt* was limited to its factual context, it could not have invalidated all applications of O.C.G.A. § 51-13-1's cap. The trial court then did the analytical work this Court demands and correctly concluded that a statutory

cap on the statutory remedy of wrongful death did not violate Georgia's jury trial right. The court also rejected Plaintiffs' other challenges, applied the cap, and reduced the judgment.

Plaintiffs now challenge that decision, asserting a bevy of procedural and constitutional objections. None should persuade.

First, Plaintiffs' waiver argument misconstrues the nature of the cap in relation to pretrial orders. The cap plays no role at trial, only becoming relevant once an offending verdict or judgment actually exists. Because the cap presents no issue for the jury's decision and does not alter the evidence before the jury, a party may assert the cap in a post-judgment motion, just like Defendants did here.

Second, Plaintiffs incorrectly assert that O.C.G.A. § 51-13-1 suffers from a severability problem, claiming that *Nestlehutt*'s holding means that every application of the cap must fail. But that reasoning flies in the face of *Turner*, which separated these applications with little trouble. Moreover, the analysis Plaintiffs propose is unnecessary where a court declares that a statute is void to the extent it violates a constitutional right, like in *Nestlehutt*. Even if a severability analysis were required, the statute can still be preserved, as applying the cap only to wrongful death awards still promotes the General Assembly's goals.

Third, Plaintiffs' jury trial right analysis is flawed. Drawing on 175 years of consistent interpretation, this Court's framework for evaluating the jury trial right asks whether Georgia juries in 1798 decided a plaintiff's claim and awarded the kind of damages a plaintiff sought. Juries in 1798 did neither for wrongful death claims; such claims are entirely a post-common-law creation. Thus, the General Assembly may limit a remedy it created, as the trial court correctly held here.

Fourth, Plaintiffs fail to show any equal protection violation. Charles Clark is not similarly situated to plaintiffs pursuing other forms of noneconomic damages, given his assertion of the distinct cause of action of wrongful death. And, in any event, the General Assembly could conceivably have determined that capping wrongful death damages, even if it was forbidden from limiting all noneconomic damages, served the legitimate government interest of promoting access to healthcare and affordable liability insurance.

Finally, Defendants ask the Court to reconsider *Nestlehutt* itself, which would negate almost all of Plaintiffs' arguments. *Nestlehutt*'s holding erroneously treats a procedural right to have juries decide the facts of a case as creating a substantive right for plaintiffs to obtain particular results from

those facts. The time has come to put *Nestlehutt* aside, and the Court should overrule it.

For these reasons, the Court should affirm the trial court's decision granting Defendants' motion to remit and entering a judgment which complied with O.C.G.A. § 51-13-1.

STATEMENT OF THE CASE

Plaintiffs' lawsuit.[1]

In May of 2019, April S. Clark had surgery to remove an ovarian cyst, during which her bowel was perforated. Over the next two days, she was treated by the nurses at Coliseum Medical Center, as well as multiple doctors, including Defendants Dr. Thomas Leigh and Dr. William Shirley. When the then-undiagnosed bowel leak became known, Clark underwent a series of operations to try and repair her bowel, primarily performed by former defendants Dr. Thomas Woodyard and Dr. John Williams. About a month later, Clark eventually succumbed to the effects of sepsis from the bowel leak.

In October 2020, Charles Clark (Clark's husband) and April D. Clark (her daughter) sued the various providers who oversaw Clark's care, including Drs. Leigh and Shirley, alleging various forms of medical malpractice. [V1 – 6-21.] Plaintiffs settled with the hospital, Dr. Williams,

[1] The underlying facts are more fully described in Defendants' brief in their cross-appeal, Case No. S26X0350.

and Dr. Woodyard, eventually proceeding to trial against only Drs. Leigh and Shirley. [See V26 – 39.] Plaintiffs targeted the 19-hour span during which Drs. Leigh and Shirley were involved, blaming them for not only for delaying Clark's return to surgery, but also her death.

The verdict.

A jury eventually returned a verdict in Plaintiffs' favor. [V20 – 74-75; V25 – 145-46.] The jury allocated 40% of the fault to Dr. Leigh, 35% to Dr. Shirley, 15% to Dr. Woodyard, and 10% to Coliseum Medical Center. [V20 – 75.] The jury awarded a total of $33,465,176, including $29.25 million for wrongful death. [V20 – 75.]

The trial court entered judgment on the verdict, apportioning damages in line with the jury's fault allocation. [V20 – 76-77.] The Court distinguished April D. Clark's recovery on behalf of her mother's estate from Charles Clark's recovery for wrongful death. [V20 – 76.] The judgment thus granted Charles Clark a recovery for wrongful death of $21,937,500.00. [V20 – 76.]

Defendants' motion to remit.

Defendants moved for a new trial, which the trial court denied. [V20 – 78-80; V26 – 12-34, 137-46.] Defendants also moved to remit and amend the judgment, asking the court to apply O.C.G.A. § 51-13-1's noneconomic damages cap to the wrongful death award. [V20 – 85-92; V26 – 32-33.] The trial court granted this motion, holding that O.C.G.A. § 51-13-1 applied to the

wrongful death following this Court's decision in *Medical Center of Central Georgia, Inc. v. Turner*, 322 Ga. 129, 917 S.E.2d 697 (Ga. 2025).[2] [V26 – 148-49.] The court then considered and rejected Plaintiffs' various arguments against applying O.C.G.A. § 51-13-1. [V26 – 149-157.]

Starting with their constitutional challenges, the court held that Plaintiffs had failed to show that capping wrongful death damages violated the jury trial right. Applying the framework laid out in *Atlanta Oculoplastic Surgery, P.C. v. Nestlehutt*, 286 Ga. 731 (2010), and *Taylor v. Devereux Foundation, Inc.*, 316 Ga. 44 (2023), the court evaluated the history of wrongful death claims in Georgia. It concluded that both the claim and kind of damages pursued here were not decided by juries in 1798, so a statutory limitation on his recovery did not offend the jury trial right. [V26 – 149-53.] Next, the court held that applying the cap to a context which *Nestlehutt* had not invalidated did not constitute judicial lawmaking. [V26 – 153-54.] Finally, the court held that Plaintiffs had failed to show that applying the cap to wrongful death violated equal protection, explaining that it is not irrational to distinguish damages awarded for wrongful death from other kinds of damages. [V26 – 154-55.]

[2] *Turner* was decided after the post-trial motions hearing. The trial court thus asked both parties to submit proposed orders to account for *Turner*'s holding. [See V26 – 148-49, 160-203, 207-33.]

The court also rejected Plaintiffs' procedural challenges. In particular, the court held that Defendants had not waived O.C.G.A. § 51-13-1's application by not asserting the cap earlier, explaining that the cap did "not present an issue for the jury to decide," but rather "define[d] the limits of the recovery a plaintiff may actually obtain following the jury's decision." [V26 – 156-57.] Thus, the need to enforce the cap only arose once a there was a "total amount recoverable" which exceeded the cap, and the cap "appl[ies] as a matter of law in that situation." [V26 – 157.] Accordingly, Defendants timely asserted the cap in a post-judgment motion. [V26 – 157.]

The trial court entered an amended judgment, revising Charles Clark's recovery for wrongful death to a total of $350,000. [V26 – 158-59.]

ARGUMENT

I. O.C.G.A. § 51-13-1's cap applied to Charles Clark's recovery of noneconomic damages for wrongful death.

As an initial matter, O.C.G.A. § 51-13-1's cap facially applied to limit Charles Clark's recovery for wrongful death. The cap's application to wrongful death is currently alive and well, because—according to this Court's decisions—that application of the cap has not been invalidated.

In 2005, the General Assembly sought to address various trends affecting healthcare access and the availability of insurance for healthcare providers through a series of reforms to Georgia's tort system. See 2005 Ga.

Laws Act 1, § 1. These reforms sought to "promote predictability and improvement in the provision of quality health care services and the resolution of health care liability claims," which would "assist in promoting the provision of health care liability insurance by insurance providers." *Id.*

One of these reforms became O.C.G.A. § 51-13-1, which limited the recovery of noneconomic damages in medical malpractice actions:

[*Omitted from this transcription: the block quotation of O.C.G.A. § 51-13-1 and the argument that follows it.*]

CONCLUSION

The Court should affirm the trial court's decision granting Defendants' motion to remit and amend and its entry of an amended judgment consistent with the limitations of O.C.G.A. § 51-13-1.

This submission does not exceed the word-count limit imposed by Rule 20.

Respectfully submitted, this 8th day of December, 2025.

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CERTIFICATE OF SERVICE

I certify that there is a prior agreement with counsel to allow documents in a PDF format sent via e-mail to suffice for service under Supreme Court Rule 14. I further certify that I have on this day served true and correct copies of the foregoing upon the following counsel of record via e-mail:

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Respectfully submitted, this 8th day of December, 2025.

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